Stability Check and Re-Assessment Triggers

Do this on Day 30, then once a month. Stable means the system works without you holding it together.

Date	Parent	Done by

A. STABLE LOOKS LIKE THIS (check what is true today)

- ☐ Medicines are taken as planned, and the routine does not depend on me remembering.
- ☐ No ER visit, hospital stay, or fall in the last 30 days.
- ☐ Every appointment is on the shared calendar, and every ride has an owner.
- ☐ Bills are paid on time, and I reviewed the statements this month.
- ☐ My parent is eating and drinking regularly, and weight and sleep are steady.
- ☐ The home hazards from Chapters 3 and 4 are fixed.
- ☐ A backup person could run a normal day from the Emergency Handoff Sheet.
- ☐ I am sleeping, working, and keeping my own appointments.

_____ of 8 checked

7-8: Stable. Keep the weekly rhythm. **5-6: Mostly stable.** Name the weak spot and give it an owner. **4 or fewer: Not yet stable.** Re-run the Seven-Area Assessment (Chapter 3) this week.

B. CHANGES THAT SHOULD TRIGGER A NEW ASSESSMENT

Trigger	Happened this month?	Date	What I did / next step
Hospital stay, ER visit, or a fall	☐		
New diagnosis, or a major medicine change	☐		
Unplanned weight loss, or eating much less	☐		
New or worse confusion, getting lost, or repeated missed medicines	☐		
New trouble with bathing, dressing, toileting, or incontinence	☐		
Unpaid bills, odd purchases, or a scam attempt	☐		
Spouse or main helper becomes ill, dies, or moves away	☐		
A paid helper quits, or the plan only works if I do extra	☐		
A driving near-miss, new dents, or getting lost behind the wheel	☐		
I am sick, exhausted, or resentful most days	☐		

One trigger: update the dashboard and tell your backup. **A hospital stay or a fall:** use the Care Roadmap on pages 3 and 4. **Two or more in the same month:** re-run the Seven-Area Assessment (Chapter 3), then the Care Path Builder (Chapter 7).

Increasing Help at Home: One Step at a Time

Add the smallest amount of help that solves the problem. Move up a step when a trigger says so, not before.

STEP	WHAT IT LOOKS LIKE	TYPICAL COST	WE ARE HERE
1	**Tools and routines.** Pill organizer, medical alert device, grab bars, night lights, meal delivery, the weekly dashboard.	Low, mostly one-time purchases.	☐
2	**Family and neighbors on a schedule.** Named owners for rides, check-in calls, and refills (Chapter 9). Backup person identified.	Your time. AARP 2025: caregivers average 26 hours a week.	☐
3	**A few paid hours.** A companion or home care aide for rides, meals, bathing, and company. Adult day program for social contact.	CareScout 2025 medians: $35 an hour; $95 a day for adult day health. Ten hours a week is about $18,200 a year.	☐
4	**Daily help.** Aide visits most days (about 20 to 40 hours a week). Possibly home health nursing or therapy after a hospital stay.	At $35 an hour, 30 hours a week is about $54,600 a year.	☐
5	**Around-the-clock care.** Overnight or live-in aides, or shifts covering all 24 hours.	Shifts at $35 an hour would be roughly $300,000 a year. Live-in arrangements can cost less. Get quotes.	☐
6	**A move.** Assisted living, memory care, or a nursing home (page 5).	See the comparison on page 5.	☐

OUR HELP-AT-HOME PLAN

Help we are adding	Who provides it	Hours per week	Cost per month	Start date

Quick math: hourly rate x hours per week x 52 / 12 = monthly cost. At $35 an hour, 10 hours a week is about $1,517 a month. Ask about minimum shifts and weekend rates.

What Medicare covers at home. Part-time or intermittent skilled care (nursing, therapy) if your parent is homebound, a doctor orders it, and a Medicare-certified agency provides it. Aide help with bathing or dressing is covered only while skilled care is also being provided. Not covered: around-the-clock care at home, home-delivered meals, and personal care when it is the only need.

If your parent has dementia: ask about GUIDE. A Medicare program, offered in all states, that gives eligible people a care navigator, caregiver training and support, a 24/7 helpline, and some respite (up to $2,500 a year, no cost sharing). Your parent generally needs Original Medicare (Parts A and B), a dementia diagnosis confirmed by a participating clinician, and to live at home. Not eligible: Medicare Advantage, PACE, hospice, or a nursing home or memory care unit.

Roadmap 1: During the Hospital Stay

The decisions that shape the next 90 days are often made in the last 48 hours of a hospital stay. Start on day one.

Hospital / unit

Date admitted

Reason for the stay

Case manager / discharge planner (name, phone)

Midnights as inpatient

Status (ask, then check)

☐ Inpatient ☐ Observation ☐ Not sure yet

FIRST 24 HOURS

- ☐ **Ask: "Is my parent admitted as an inpatient, or in observation?"** Observation time does not count toward the three-day inpatient stay Medicare requires before it will cover skilled nursing facility care.
- ☐ Give the team the Quick Sheet, medicine list, and the name of the health care agent (binder pages 2 and 3). Say what is normal for your parent so changes get noticed.
- ☐ Get the name and number of the case manager or discharge planner. Bring glasses, hearing aids, and dentures.

EVERY DAY (three minutes: who said what, and what is still open)

Date	Who I spoke with	What they said	Open question

BEFORE DISCHARGE (start asking about this on day two)

- ☐ Ask for the expected discharge date and where your parent is going (home, home health, rehab). Ask early, and ask again each day.
- ☐ **You have the right to a fast appeal.** Hospitals should give an "Important Message from Medicare" within two days of admission. To appeal a discharge you think is too soon, contact the quality reviewer (QIO) named on that notice no later than the planned discharge day.
- ☐ Compare the final medicine list line by line with the bottles at home: what is new, changed, stopped.
- ☐ Follow-up appointment booked, with a date, and you know who scheduled it.
- ☐ Equipment and home health arranged before leaving, not after.
- ☐ Written instructions in hand, plus a number to call with questions at night.
- ☐ **Teach-back:** you have shown the nurse that you can do each new task (wound care, injections, transfers, a pill schedule).

Fast-appeal contact (QIO name and phone from the notice)

Discharge to (home, home health, rehab facility)

New, changed, and stopped medicines

Follow-up appointment

Equipment ordered

Roadmap 2: Rehab and the First 30 Days Home

Most of the risk comes after discharge. Plan the first three days and the first month before you leave.

Why the first 30 days matter. CMS has reported that about one in five Medicare patients discharged from a hospital were readmitted within 30 days (older data). A clear medicine list, a booked follow-up visit, help in place at home, and someone who knows the warning signs all help reduce the odds of a return trip.

If the next step is a skilled nursing facility (SNF). Medicare Part A may cover rehab after at least three consecutive days as a hospital inpatient (not observation). Coverage requires a daily skilled need and lasts up to 100 days per benefit period. **2026:** days 1-20 cost $0 after the $1,736 hospital deductible; days 21-100 cost $217 a day; after day 100 you pay all costs. Custodial care alone is not covered. Some Medicare Advantage plans waive the three-day rule or charge copayments; ask the plan.

If the next step is home health. Medicare covers part-time skilled nursing and therapy at home when your parent is homebound, a doctor sees them and orders it, and a Medicare-certified agency delivers it. Covered home health services cost nothing; equipment is typically 20% after the Part B deductible. It does not cover around-the-clock care or personal care as the only need. Before care starts, ask the agency what Medicare will and will not pay.

COMPARING REHAB FACILITIES (ask each one)

Facility	Distance	Therapy hours / day	Bed available?	Night / weekend staffing, other notes

- ☐ Ask: how and how often will they update the family, and who is our one point of contact?
- ☐ Ask: when does discharge planning start? (A good answer: from the first days.)

FIRST 72 HOURS HOME

- ☐ Medicine organizer filled from the **final** list
- ☐ Follow-up visit on the calendar, ride arranged
- ☐ Food in the house; clear paths; night lights on
- ☐ Someone with your parent overnight, or a plan
- ☐ Warning signs list by the phone; Quick Sheet on the fridge
- ☐ Home health or therapy start dates confirmed

FIRST 30 DAYS

- ☐ **Week 1:** go to the follow-up visit with the dashboard and questions
- ☐ **Week 1:** pharmacist reviews every medicine, new and old
- ☐ **Week 2:** therapy attended; walking and balance checked
- ☐ **Weeks 3-4:** weekly review running; backup person briefed
- ☐ **Day 30:** run the Stability Check (page 1)
- ☐ **Day 30:** update the binder and handoff sheet

WARNING SIGNS AFTER DISCHARGE (same colors as the decision trees)

RED: Call 911. Stroke signs (sudden loss of balance, vision change, face drooping, arm weakness, speech trouble), chest pain or pressure, severe trouble breathing, cannot be woken, severe bleeding.

YELLOW: Call the doctor or the discharge nurse line today. New or worse confusion, fever, a fall, new pain, redness, swelling, or drainage at a wound, not eating or drinking, trouble urinating, or a medicine that seems to be causing problems.

Questions for the follow-up visit

Care Options Compared

Home care, assisted living, memory care, nursing home, and family care. Medians are national; your area will differ.

	Home care (paid aides)	Assisted living	Memory care	Nursing home (skilled nursing)	Family care (you and family)
What it is	Aides come to the home for company, meals, bathing, and errands. Nurses and therapists for skilled needs.	Apartment-style residence with meals, help with daily activities, and staff on site.	Secured residence or unit with staff trained in dementia and structured routines.	24-hour nursing and personal care, for short-term rehab or a long-term stay.	You and others provide the care, with or without paid help.
Fits best when	Your parent wants to stay home, needs help with some daily tasks, and the home is (or can be made) safe.	Daily help is needed but not constant nursing, or home is isolated or unsafe.	Memory loss brings wandering, safety, or agitation that home or assisted living cannot manage.	Needs 24-hour nursing or complex medical care that cannot be given safely elsewhere.	Needs are modest and the family can keep up the hours without harming health or work.
Typical cost (national medians)	$34 to $35 an hour (A Place for Mom 2026; CareScout 2025).	About $5,400 to $6,200 a month (A Place for Mom 2026; CareScout 2025: $74,400 a year).	About $6,700 a month (A Place for Mom 2026).	$315 a day semi-private ($114,975 a year); $355 a day private ($129,575) (CareScout 2025).	No fees, but about 26 hours a week (AARP 2025) and roughly $7,200 a year out of pocket (AARP).
Who may pay	Mostly private pay or long-term care insurance. Medicare pays only for part-time skilled home health when homebound.	Mostly private pay or long-term care insurance. Medicaid help varies by state. Medicare does not pay for custodial care.	Same as assisted living, at a higher price. Medicare does not pay for custodial care.	Medicare covers short rehab stays only (up to 100 days; $217 a day from day 21 in 2026). Long stays: private pay, insurance, or Medicaid if eligible.	Your own money. Ask your Area Agency on Aging about respite and caregiver support.
Watch for	Costs climb quickly as hours rise. Agency or hiring directly? Who covers sick days?	Move-in contracts, extra fees as needs grow, and what level of need forces a move out.	Night staffing, dementia training, wandering safety, and how fees change over time.	Quality varies by facility. Check inspection reports and staffing, and understand discharge rights.	Your health, work, and family. Alzheimer's Association: 59% of dementia caregivers report high or very high emotional stress.
Ask first	Rate and minimum shift? Backup if the aide is out? Dementia training?	What is included? What triggers extra fees or a required move?	Staff-to-resident ratio at night? How do you handle exit-seeking and agitation?	Therapy hours? One point of contact? What do state inspections show?	Who is the backup? What is our limit? What would make us add help?

OUR SHORTLIST (visit at least two; compare the same things)

Option or place	Monthly cost	Distance	Tour date	Questions still open

Prepare Without Deciding Too Soon

Agree on the signs now. Decide later, with the facts in front of you.

A. IS LIVING ALONE STILL SAFE? (check what you have seen in the last 60 days)

- ☐ A fall or near-fall
- ☐ Could not get up, or could not call for help
- ☐ Missed or doubled medicines
- ☐ Little or spoiled food in the house
- ☐ Unplanned weight loss
- ☐ Burns, a stove left on, or other fire risk
- ☐ Wandering or getting lost
- ☐ Unpaid bills or scam victim
- ☐ Unsafe at night (bathroom trips, stairs)
- ☐ Refuses help while the risk is rising

0 or 1 checked: keep going and watch. **2 or 3:** step up help at home (page 2) and ask the doctor for an assessment. **4 or more, or any one of these: cannot get up after a fall, wandering, a fire risk:** meet this week about a safer arrangement. About one in four adults 65 and older reports a fall each year, according to the CDC.

B. DECISION TRIGGERS (agree now, decide later)

Decision	The sign we agreed on (something observable)	Who decides / who weighs in	What we gather now (no commitment)	Revisit by
Stop driving				
Add paid help at home				
Move to assisted living				
Memory care or nursing home				
Take over bills (financial POA)				

DO NOW
- Tour two places and collect price lists, fee schedules, and contracts. No deposits.
- Ask what level of need forces a move out.
- Confirm the health care directive and powers of attorney are current.
- Find out what long-term care insurance, VA benefits, or Medicaid might cover.
- Keep the binder and dashboard current.

DON'T DO YET
- Sign a contract or pay a deposit you cannot get back.
- Sell the house or retitle assets without an elder law attorney. Ask about Medicaid look-back rules first.
- Promise your parent "never a facility" or "always home." Say: "We will keep making home work, and we will check together."
- Decide alone what the family agreed to decide together.

Protect the Caregiver: Health, Work, and Family

Your parent's plan fails if yours does. Put your own limits on paper, just like theirs.

You are not the only one. Alzheimer's Association (2025): 59% of family caregivers of people with dementia rate their emotional stress as high or very high; 57% arrived late, left early, or took time off work, and 16% took a leave of absence. AARP (2025): emotional stress among caregivers has risen since 2020.

A. MY WARNING SIGNS (check what is true this month)

- ☐ Not sleeping, or sleeping far too much
- ☐ Skipping my own appointments
- ☐ Getting sick more often
- ☐ Short-tempered with my parent, partner, or kids
- ☐ Dreading the phone
- ☐ Resentment or hopelessness
- ☐ Eating or drinking more or less than usual
- ☐ My work is slipping

Three or more checked: change something this week. If caregiving ever feels hopeless, talk to your own doctor. In the United States you can call or text 988 at any hour.

B. MY LIMITS

The most hours a week I can give without harm

Things I will not do (for example, overnight care, bathing)

What I need from my family

My own appointments this quarter (dates)

C. RESPITE: SCHEDULED, NOT HOPED FOR

Date	Who covers	For how long	What I will do (rest, not errands)

D. WORK

Family and Medical Leave Act (FMLA). If your employer is covered (generally 50 or more employees within 75 miles) and you have worked there 12 months and 1,250 hours in the past year, you can take up to 12 weeks of unpaid, job-protected leave in a 12-month period to care for a parent with a serious health condition. Leave can be taken in blocks or on a reduced schedule when medically needed, and your employer must maintain your health insurance. Parents-in-law do not qualify. Ask HR to confirm.

- ☐ Asked HR about FMLA, flexible hours, and any employee assistance program
- ☐ Got my parent's doctor to complete the leave paperwork if needed

HR contact and what I learned

Talked with my partner/children about the time this takes (date)

90-Day Family Action Plan

Three phases. Every action gets one owner and one date. Add your own in the blank rows.

Start date (Day 1)	Parent	Plan lead

DAYS 1-30: STABILIZE AND TEST

Action	Owner	Due	Done
Run the Stability Check (page 1); fix the weakest area first			☐
Binder pages 2-6 filled in; Quick Sheet on the fridge			☐
Weekly dashboard running; one owner on every task			☐
Emergency Handoff Sheet written and read back			☐
Family meeting #1 held (page 9)			☐
Book my own checkup and one respite block (page 7)			☐
			☐
			☐

DAYS 31-60: STRENGTHEN

Action	Owner	Due	Done
Try paid help for a few hours a week (page 2)			☐
Get price lists and tour two options, no commitment (page 5)			☐
Meet an elder law attorney: documents, Medicaid, paying for care			☐
Ask the doctor for a fall-risk review and a memory check if triggers fit			☐
Review Medicare plan and drug coverage when enrollment opens (fall: Oct. 15 to Dec. 7)			☐
Agree on decision triggers (page 6)			☐
			☐
			☐

DAYS 61-90: PREPARE AND REVIEW

Action	Owner	Due	Done
Re-run the Seven-Area Assessment (Ch. 3) and Care Path Builder (Ch. 7)			☐
Re-run the Stability Check; compare with Day 30			☐
Family meeting #2: choose no change, add help, or start a transition (page 9)			☐
Update the binder, handoff sheet, and decision triggers			☐
Check my own warning signs and limits (page 7)			☐
Set the date of the next 90-day review			☐
			☐
			☐

Family Meeting Agenda (60 Minutes)

Use at Day 30 and Day 90. Facts first, feelings second, decisions last.

Date

In person / video / phone

Who is attending

Facilitator (not the main caregiver, if possible)

Note-taker, and who sends the notes

1. FACTS FIRST (10 min) Read the dashboard summary and the Stability Check score aloud. No opinions yet.

2. WHAT CHANGED (10 min) Which triggers on page 1 happened? Any hospital stay, fall, or new diagnosis?

3. HOW IS EVERYONE? (10 min) Each person says how they are doing. No fixing, no comparing who does more.

Notes from items 1 to 3

4. DECISIONS ON THE TABLE (15 min) Only decisions whose page 6 triggers were reached.

Decision	Options we considered	Who decides	What we decided

5. ASSIGN OWNERS (10 min) One name and one date per action. "We" is not an owner.

Action	Owner	Due	Done
			☐
			☐
			☐
			☐

6. CLOSE (5 min) Confirm the next meeting date. Say one thing that went well.

Next meeting (date, time, who hosts)

Something that went well

Ground rules. One person talks at a time. The question is what your parent needs, not what is fair between siblings. If your parent can take part, invite them for the part that concerns them, and let them decide what they are able to decide (Chapter 6).